

SCALP SYMPTOMS

- If hair loss with no rash/itch → 81.
- Is there a rash or only an itch?

Itch without rash

Severe itch with lice or white eggs. May have small red bites on back of neck.

Lice likely

- Apply **permethrin 5%** lotion to towel-dried or dry hair:
 - Using normal comb, comb into hair to ensure whole scalp is covered and hair is saturated.
 - Then using fine lice comb, remove lice and eggs from hair in sections, combing away from scalp.
 - Rinse lice comb in hot water in white bowl or wipe on white tissue between strokes to identify black lice.
 - Rinse off after combing (up to 1 hour).
 - Repeat every 5 days for 3 weeks. Lice should get smaller with each treatment. If not, check patient is applying permethrin correctly.
 - Avoid broken skin/eyes.
- Wash clothes and linen used in past 2 days in very hot water.
- Treat household contacts.
- Consider shaving head only if acceptable to patient.

Fine, white flakes on hair and clothing

Dandruff likely

- Apply **selenium sulphide 2.5%** suspension:
 - Lather on scalp.
 - Rinse off after 10 minutes.
 - Use weekly until better, then every second week.

Rash with or without itch

Scaly patches

Red/pink patches with fine greasy scales. May also occur between eyebrows, in nose folds, behind ears. Usually itchy.



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Seborrhoeic dermatitis likely

- If extensive, test for HIV > 110.
- Apply **selenium sulphide 2.5%** suspension:
 - Lather on scalp.
 - Rinse off after 10 minutes.
 - Use weekly until better, then every second week.
- For skin: apply **hydrocortisone 1%** cream twice a day. Once improved, reduce to once or twice a week as needed.
- If poor response or severe, apply instead **betamethasone 0.1%** ointment once a day for 7 days (avoid face, neck and creases).
- If no response within 2 months, refer.

Well-defined, raised plaques covered with silvery scale. Often on knees, elbows, lower back, scalp. May have pitted nails.



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Psoriasis likely

- Refer to specialist to confirm diagnosis.
- While waiting for appointment:
 - Moisturise skin with **emulsifying ointment (UE)** twice a day.
 - Apply **betamethasone 0.1%** ointment twice a day. Once improving, apply instead **hydrocortisone 1%** cream twice a day, then reduce to once a day. Stop as soon as better or apply **liquor picis carbonis (LPC) BP 5%** ointment once a day.
 - Encourage to expose skin to sunlight before 10am or after 3pm for up to 30 minutes per day.

Redness, swelling and burning/itching after recent use of hair product. May have blisters.

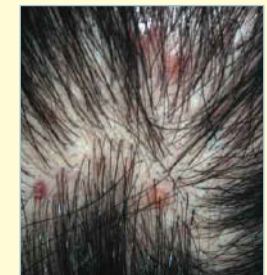


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Contact dermatitis likely

- Identify and advise patient to avoid cause.
- Moisturise skin with **emulsifying ointment (UE)** twice a day.
- Apply **betamethasone 0.1%** ointment twice a day. Once improving, apply instead **hydrocortisone 1%** cream twice a day, then reduce to once a day. Stop as soon as better.
- If pus or yellow crusts, treat for infection:
 - Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days.
 - If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- If no better, refer.

Red pimples, pustules or nodules around hair follicles



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Folliculitis likely

- Advise to wash with soap twice a day.
- Wash scalp with **chlorhexidine scrub** once a day until lesions resolve.
- If infection deep, extensive, recurrent or no response to above treatment:
 - Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days.
 - If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days
- Test for HIV > 110.

If diagnosis uncertain, discuss/refer.

¹History of angioedema, anaphylaxis or urticaria.